Hello Heart

Re: RFI – Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

On behalf of Hello Heart, we appreciate the opportunity to respond to the Department of Health and Human Services' (HHS) Request for Information on accelerating the adoption and use of artificial intelligence (AI) in clinical care.

Hello Heart is a digital health company focused on improving cardiovascular and metabolic outcomes at scale. We partner with employers, health plans, healthcare professionals, and health systems to deliver personalized, technology-enabled care that combines connected devices, longitudinal data analysis, behavioral science, and clinical expertise. Cardiovascular disease remains the leading cause of death and one of the most significant drivers of health care costs in the United States. AI, when responsibly developed and deployed, can materially improve early risk detection, care navigation, and chronic disease management in ways that reduce cost and improve outcomes.

However, several systemic barriers continue to slow innovation and responsible adoption of the use of AI in clinical care.

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1. Biggest Barriers to Private Sector Innovation and Adoption of AI in Clinical Care

a. Data Access, Infrastructure, and Patient Control

AI systems are only as strong as the data they are built upon. In cardiovascular care, meaningful insights require longitudinal, multimodal data claims, labs, vitals, pharmacy records, and patient-reported information. While policy progress has been made under the 21st Century Cures Act and the ONC interoperability framework (45 C.F.R. Part 170; 45 C.F.R. Part 171), operational barriers remain significant.

Health data are still fragmented, difficult to aggregate, and burdensome to access in real time. To unlock AI's full potential, HHS should prioritize building a true data "superhighway" that enables secure, standardized, API-driven exchange. Critically, this infrastructure must place meaningful, operational control in the hands of patients. Patients, not providers or institutions, should have the final authority to determine what data are shared, with whom, and for what purpose, including with the digital health and AI solutions they choose to use.

Reducing operational friction in data sharing will directly accelerate innovation in prevention, early detection, and personalized care.

b. Regulatory Ambiguity Around FDA Thresholds

There remains insufficient clarity regarding when AI functionality crosses the threshold into regulated medical device territory under the Federal Food, Drug, and Cosmetic Act and 21 C.F.R. Parts 801, 807, and 860.

Many AI-enabled clinical decision support (CDS) tools, including those that synthesize data, stratify risk, or provide care guidance, augment clinical judgment rather than independently diagnose or prescribe. Yet uncertainty persists regarding whether such tools require a multi-year premarket review process.

Regulatory predictability is essential for sustained investment and responsible scaling. As such, we respectfully urge HHS, in coordination with the FDA, to:

- Provide clearer operational guidance distinguishing supportive AI from autonomous medical decision-making;
- Clarify application of Section 520(o)(1)(E) of the FD&C Act to modern AI-based CDS tools;
- Establish risk-based, streamlined regulatory pathways for lower-risk AI systems that enhance, not replace, clinical oversight.

c. Defensive Health Care and Fear of Litigation

The U.S. health care system is structured to minimize liability exposure. Even where evidence demonstrates that AI performs as well as or better than human clinicians in defined use cases, providers may hesitate to adopt such tools out of concern for litigation risk.

Without defined protections or safe harbor frameworks, clinicians and institutions may avoid using AI, even when doing so could improve patient safety. HHS should explore mechanisms that provide reasonable liability protections for the appropriate use of validated AI systems that meet recognized safety and transparency standards.

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2. Regulatory, Payment Policy, and Programmatic Changes HHS Should Prioritize

a. Strengthen Patient-Directed Interoperability and Data Rights

HHS should continue strengthening enforcement and operationalization of:

- 42 U.S.C. § 300jj-52 (Information Blocking);

- 45 C.F.R. Part 171;
- 45 C.F.R. Part 170 (ONC Certification Program).

In addition, HHS should explicitly reinforce that patients have the right to direct their data to third-party applications of their choosing without unnecessary provider-imposed friction, delay, or discretion. Patients should not be required to rely on provider-mediated approvals to share their own health information with digital health tools they select.

Reducing administrative and technical burdens in accessing standardized FHIR-based data will allow AI systems to generate more accurate risk assessments and care recommendations. Empowering patients to control and share their data seamlessly across platforms will also accelerate innovation in preventive care and chronic disease management. A patient-centered interoperability framework, where individuals can easily and comprehensively share their complete health record with trusted AI-enabled solutions, will expand competition, improve care coordination, and promote innovation.

b. Align Payment Models With AI-Enabled Value

AI is particularly powerful in value-based care environments, where prevention, early intervention, and risk stratification drive measurable savings. CMS should explore ways to recognize AI-enabled clinical decision support, navigation, and risk management within existing value-based arrangements and quality programs.

Clear recognition of AI-enabled care coordination, navigation, and decision support within reimbursement frameworks would accelerate responsible adoption. When AI demonstrably improves HEDIS and STARS performance, reduces hospitalizations, avoids unnecessary and/or duplication of services and/or lowers total cost of care, payment models should reflect those improvements.

c. Provide Clear Guardrails Without Overregulation

HHS should ensure that regulatory oversight is proportionate to risk. Subjecting supportive, lower-risk AI tools to lengthy approval processes will slow beneficial innovation without materially improving safety. Clear guidance will reduce uncertainty while maintaining appropriate safeguards.

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3. Novel Legal and Implementation Issues

AI deployed within clinical workflows raises questions around liability allocation, transparency, and accountability.

a. Clear Accountability Frameworks

Developers, providers, and institutions require clarity regarding responsibility when AI augments clinical decision-making. Absent defined frameworks, adoption will remain cautious and uneven.

To address this, HHS could support development of model governance structures or voluntary safe harbor programs for AI systems that meet defined safety benchmarks and transparency criteria.

b. Clarifying FDA Jurisdictional Boundaries

As noted, clearer thresholds distinguishing regulated devices from supportive tools are essential to avoid unnecessary delays in deploying beneficial technologies. Developers need operational clarity to avoid over-regulation of tools that merely support clinical reasoning.

c. Protecting Patient-Directed Data Use

As patients gain greater authority to direct their data to AI-enabled solutions, clear standards should ensure that their choices are respected and that providers cannot unreasonably restrict patient-authorized data exchange. Strengthening patient data agency will reduce fragmentation and enable more accurate, personalized AI-driven insights.

d. Encouraging Evidence-Based Adoption

Where AI systems meet or exceed clinician-level performance in specific domains, policy frameworks should support their use. Defensive medicine should not prevent deployment of tools that can demonstrably reduce error and improve outcomes.

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4. AI Evaluation Methods and the Role of HHS

Responsible AI deployment requires rigorous evaluation before and after launch.

Effective approaches include:

- Expert clinical validation during model development, including structured scoring and incorporation of gold-standard examples;
- Automated evaluation frameworks that continuously assess safety, accuracy, and appropriateness at scale;

- Ongoing post-deployment sampling and human review to ensure sustained quality.

HHS could play a transformative role by developing a standardized AI safety evaluation framework or benchmarking model that developers could run their systems against prior to deployment. An HHS-backed safety certification or "seal" would significantly increase provider and patient trust.

Mechanisms such as cooperative agreements, grants, or public-private partnerships would be effective vehicles for developing and operationalizing such standards.

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5. Accreditation and Certification

HHS can accelerate adoption by supporting:

- Voluntary AI safety certification programs;
- Standardized transparency and reporting frameworks; and
- Industry-led testing consortia operating under clear federal guidance.

An HHS-developed safety evaluation model, coupled with public certification pathways, would provide assurance to providers and patients and accelerate adoption.

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6. Performance and High-Impact Use Cases

AI has demonstrated strong performance in domains requiring data synthesis and clinical judgment, including risk prediction, triage, and chronic disease management. In cardiovascular and metabolic care, AI can identify risk patterns earlier than traditional workflows and support personalized interventions.

AI is particularly promising for:

- Early identification of hypertension, diabetes, obesity, hyperlipidemia and cardiovascular risk;
- Personalized education and behavioral support;
- Clinical data summarization;
- Care navigation and referral optimization; and
- Preventive care adherence.

While procedural robotics remains evolving, AI-based decision support should increasingly serve as a first line of defense in judgment-based clinical scenarios that can lead to improvement in health outcomes.

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7. Organizational Decision-Makers and Administrative Hurdles

Within health care organizations, the Chief Medical Officer (CMO) or VP Medical Affairs (VPMA) typically holds decisive influence over clinical AI adoption. Without clinical leadership endorsement, implementation is unlikely to succeed.

Primary hurdles include:

- Legal and compliance uncertainty;
- Procurement complexity;
- Liability concerns; and
- Cultural resistance driven by safety perceptions.

Clear regulatory guidance and validated safety frameworks will directly influence executive-level adoption decisions.

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8. Interoperability and Market Acceleration

Enhanced interoperability would expand opportunities across nearly all domains of AI-enabled care. Standardized exchange of structured clinical data, remote monitoring data, pharmacy records, and patient-reported outcomes is foundational to innovation.

True interoperability must mean patient-directed interoperability. When patients can seamlessly aggregate and authorize use of their complete health data across settings and solutions, AI tools can deliver more accurate, preventive, and personalized care.

Continued enforcement and evolution of FHIR-based exchange standards will materially accelerate both research and commercialization.

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9. Patient and Caregiver Perspectives

Patients and caregivers consistently seek:

- Safety and trust in AI-enabled care;

- Clear guidance on what care to seek and when;
- Administrative simplification, including consolidated medical summaries; and
- Personalized education tailored to their specific conditions.

Trust is central. Government-backed validation frameworks and transparent evidence demonstrating AI performance relative to human decision-making will be essential to broad acceptance.

At the same time, provider resistance, often framed around safety, may slow progress. Public, evidence-based evaluations can help bridge that gap. Transparent evaluation standards and public evidence demonstrating AI performance relative to human decision-making will be essential to overcoming skepticism.

Patients also increasingly expect control over their own health information. Empowering individuals to determine how their data are shared, and ensuring that their choices are honored, will be foundational to trust in AI-enabled care.

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10. Priority Research Areas

HHS should prioritize:

- Comparative studies evaluating AI decision-making against clinician performance;
- Research on effective human-AI collaboration models;
- Longitudinal outcome studies measuring quality and cost impact; and
- Standardized safety benchmarking methodologies.

Publishing rigorous, peer-reviewed evidence demonstrating where AI meets or exceeds human-level performance will be instrumental in building public and provider trust and accelerating responsible adoption.

Conclusion

AI has the potential to meaningfully improve cardiovascular and metabolic outcomes, enhance clinical decision-making, reduce administrative burden, and lower total cost of care. Realizing that potential requires improved data infrastructure, clear and proportionate regulation, aligned payment incentives, and trusted safety validation frameworks.

Most critically, the future of AI-enabled clinical care must be built on a patient-directed data ecosystem. Patients should have clear, enforceable authority to determine what health information is shared, with whom, and for what purpose, including with the digital health and AI solutions they choose to use.

By strengthening patient-controlled interoperability, clarifying FDA regulatory thresholds, supporting appropriate liability guardrails, and establishing standardized AI safety evaluation frameworks, HHS can accelerate responsible innovation while reinforcing trust, privacy, and clinical integrity.

We appreciate the opportunity to provide input as Hello Heart stands ready to collaborate with HHS, health plans, healthcare professionals and health systems to advance responsible, evidence-based AI adoption that improves patient outcomes and strengthens the health care system.

Respectfully submitted,

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Hello Heart